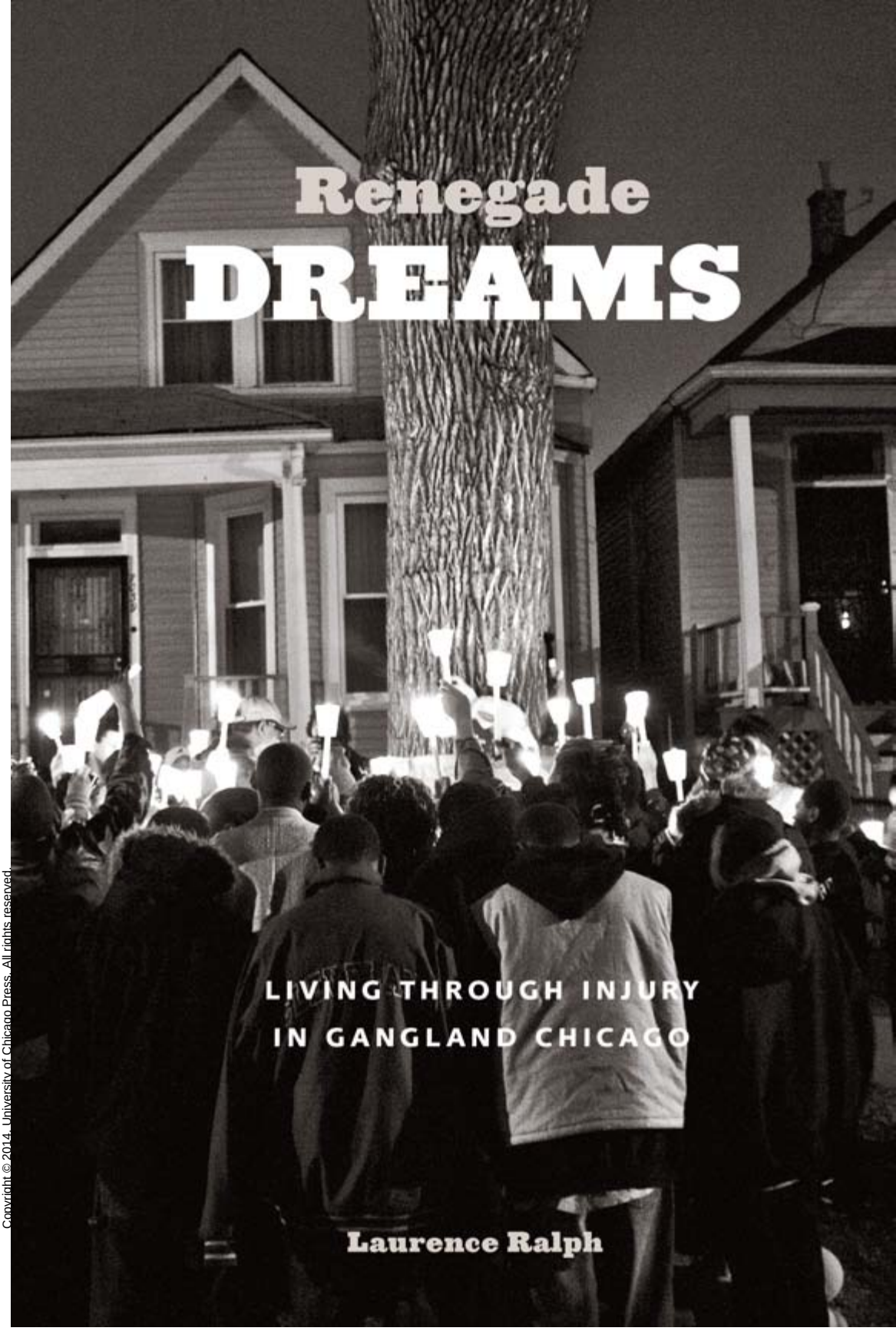

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CHAPTER FIVE

Disease

OR,

How a Will to Survive Helps the Healing

In cellblock 3B, Amy O'Neal is one of a half-dozen girls gathered around a table awaiting the start of Bible study. I sit across from her. This is December 2007, at the female wing of the West Side Juvenile Detention Center. Monica Namba, a twenty-two-year-old volunteer for Eastwood Community Church's outreach ministry, has brought freshmen from a nearby Bible college to the detention center for the first time. She is introducing them to Amy, one of her favorite inmates. "Amy has HIV," Monica says to the college group. "But she has the strongest spirit."

The Bible study begins abruptly when Monica instructs everyone to turn to John, chapter eight, verse four. Amy reads:

> "'Teacher,' they said to Jesus, 'this woman was caught in the very act of committing adultery. In our Law Moses commanded that such a woman must be stoned to death. Now what do you say?' They said this to trap Jesus, so that they could accuse him. But he bent over and wrote on the ground with

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> his finger. As they stood there asking him questions, he straightened up and said to them, 'Whichever one of you has committed no sin may throw the first stone at her.' Then he bent over again and wrote on the ground."

At the end of the sentence the next young woman around the table begins reading in seamless transition:

> "When they heard this, they all left, one by one, the older ones first. Jesus was left alone, with the woman still standing there. He straightened up and said to her, 'Where are they? Is there no one left to condemn you?' 'No one, sir,' she answered. 'Well, then,' Jesus said, 'I do not condemn you either. Go, but do not sin again.'"

Every few months, Monica makes sure that every incarcerated young woman who's willing recites this verse; doing so offers different sets of teenagers who find themselves in the same cell a steadfast message of redemption.

After Bible study, the guards suggest that the teenagers spend the rest of the recreational period, about fifteen minutes, working on their creative projects. Some write poems or songs, others pen letters to loved ones. Monica encourages me to talk to Amy—the least shy of the bunch. I approach Amy, her head resting on the table, a tattoo of a woman's face visible on her forearm. Above the likeness of the woman, the words "R.I.P Sue-Anne Green" are written in an Old English font. As I'm studying the tattoo, Amy suddenly shoots up: an elderly nurse has called her name from a clipboard. Immediately, Amy begins a seven-pill regimen, the nurse checking under her tongue after each one she swallows. "That a girl," the nurse says.

Meanwhile, Monica rounds up the Bible college students. I leave the detention center with them, unaware that over a year would pass before I would have my first conversation with Amy.

Leaders of the Eastwood Community Church's outreach program have a seemingly straightforward goal: Convert nonbelievers into followers of Jesus Christ. For them, the fact that a teenager like Amy attends

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Bible study every week suggests that one day she'll dedicate herself to personal transformation. And yet embedded in the church's outreach initiatives are a number of ideas about sin and healing (and injury and redemption) that can easily become conflated. By examining this outreach within the context of the church's community mission, we can begin to understand the ways in which disparate institutional notions of healing bleed into one another.

Members of the church's outreach ministry equate HIV with sexual deviance and sexual deviance with sin. And sin, in their view, is the impetus for moral transformation as well as social reform. It shouldn't surprise us that in Eastwood—as in many urban communities—rates of diseases and addictions are disproportionately high; for this reason, finding ways to contain their influence becomes the basis for both governmental intervention and economic investment. For the church, though, addressing disease and addiction now means grappling with new interpretations of how the body heals from injury. No longer, from a medical standpoint, is healing defined as either a full recovery or a terminal diagnosis (or, from a religious perspective, bodily transcendence). Rather, healing is an ongoing process. Increasingly, the concept of healing is associated with those who must manage chronic illness.[1]

One of the ways that the church helps patients heal is by fostering an environment that prizes willingness—a willingness to remain faithful or sober or loyal to an antiretroviral drug treatment regimen. The challenge that this church encounters is that some Eastwoodians do not merely wish to transcend addiction and disease through religion and leave their past behind. Residents like Amy dwell within a space of injury and harness the resolve it affords. I would come to learn that Amy, for instance, doesn't merely wish to develop a religious-based will, but an extra-religious "renegade will," akin to the brazen attempts to escape life's constraints already discussed.

The "renegade will" plays a vital role in helping leaders of both religious and medical institutions understand the ways that injury is linked to urban poverty; and in a community like Eastwood, where many are susceptible to the same kinds of diseases and addictions, narrating one's experiences of injury become a vital resource. In what follows, we'll see

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that when healing translates to the management of myriad injuries, the sixteen-year-old HIV-positive drug addict with the tattoo on her forearm is well positioned to teach us about resiliency.

Mark is a health worker. He is employed by Healing Hearts, a nonprofit organization that provides support to HIV-positive teenagers. Three weeks after Amy's release from the detention center, Mark was assigned to her case. In the year that they have known each other, Mark has built a strong relationship with her. Case in point: I first met Mark, a gay black man in his forties, at Eastwood Community Church when he came to visit Amy. "You know I love Amy, right?" he said. "'Cause I haven't stepped into a church in years."

Although he considers himself a Christian, Mark hasn't felt comfortable in a church for some time, given that most of those he has attended associate homosexuality with sin. Months ago, however, Mark broke his promise to not step foot in a religious institution until he is free to marry in one—this was after Amy convinced him to listen to her speech at a local church. That she wants him to hear her story is a feat in itself, Mark explains, because they didn't always get along. The breakthrough came when Mark proved himself to be more than just Amy's health worker—specifically, the day he confided in her. Like Amy, Mark is living with HIV and has been for five years. His personal stake in Amy's health, he confesses, stems from seeing many young people die.[2]

HIV-positive teenagers like Amy, many of whom have difficulty adhering to a treatment regimen, are extremely vulnerable for premature death. Mark explains: "They get sick. They don't tell anyone. Now, that's problematic." He angles forward in his seat. "But that's not even the main set of issues we're worried about. Most of the time, they don't have any symptoms at all. Then they stop taking their medication altogether. By the time they get *really* sick, it's too late to prolong their lives."

Mark pauses for a moment, shaking his head contemplatively before continuing: "And don't let me get started on the drugs. Now that's adding insult to injury.[3] It's hard enough to get a drug addict to tell you what day of the week it is, much less the hour of the day. And that's what you gotta know: It's extremely important to take your pills at the right time—and to make appointments."[4]

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Mark fears that combined with the threat of relapse with heroin addicts like Amy, a high-risk lifestyle associated with drug abuse will justify the withdrawal of antiretroviral drugs. Over the next several months, Mark will spend time helping Amy develop the skill for identifying which drugs relate to which physical experiences. He will encourage her to speak with her physician about the effects that the antiretroviral cocktail therapy will have on her bodily chemistry, to schedule doctor's appointments so that her physician will be able to adjust the regimen relative to the progression of the disease, and to monitor her exercise and eating habits. For Amy, maintaining a healthy diet, taking her medication regularly, going to sleep at the right time, learning to be alert to her physical symptoms, and scheduling regular medical appointments are all vital to maintaining the kind of lifestyle that could not have been imaginable a few decades ago with patients diagnosed with HIV.[5]

Over the past several years, antiretroviral drugs (drugs that work to maintain function of the immune system by preventing infections and thus extend an HIV patient's life span) have headlined the burgeoning research on HIV and AIDS. In both Western and non-Western contexts, antiretroviral drugs have dramatically changed the social landscape in disease-infected regions: present-day generations are able to take part in society in a way that was previously unthinkable. As social scientists have pointed out, however, having the drugs is merely the first step; even armed with this medicine, the barriers for treating populations vulnerable for acquiring HIV are substantial, as are the risks associated with improper use. The assumption that a supply of antiretroviral drugs is a panacea for infected populations, in other words, has proven to be overly optimistic.[6]

This new era of healing through the management of chronic illness has made the issue of adherence ever more salient. Non-adherence poses health risks not only for the individual, but for the entire community. If a patient misses doses of an antiretroviral drug regimen, drug resistance can develop. This term refers to strains of the virus initially resistant to the drug treatment that eventually become dominant in an infected person's bloodstream. The prevalence of drug-resistant strains makes it more difficult to treat the infected individual, as well as anyone else that

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person may infect. Doctors stress that those who initiate antiretroviral therapy must be prepared to commit to lifelong treatment.[7]

"Folks have got it all wrong," Mark says. "They think people find out they're HIV positive, wake up the next morning, and then—*boom*—like magic, start living a disciplined life. Nope. Truth is—being *willing*—being willing to adhere," he repeats, "is just as important as adherence itself."[8]

Healing, as defined by Mark, resides at the intersection between clinical and moral understandings of illness. Although Mark is trained to understand the progression of HIV, like many community experts his effectiveness is limited by the fact that the willingness he seeks to inspire must take hold in an African American Eastwood, a neighborhood whose sizable population of young black inhabitants are thought to lack moral resolve. The fact that they must deal with a community that is characterized by its "reckless will" is what pairs the priorities of Mark's institution, Healing Hearts, with that of Pastor Tim's.[9]

Many times I have witnessed the dismay on Pastor Tim's face when he opens the *Eastwood Gazette* to dire HIV statistics in his area.[10] He explains the numbers as a result of the infected persons buying into "street" culture. But unlike many Eastwoodians who locate danger in an abstract notion of inner-city dysfunction, Pastor Tim does not just complain. He feels it his duty to take a hands-on approach and help develop strength of character in Eastwood's youth, fortifying their will with religious resolve.

Each week Pastor Tim traverses James Street and tries to convince the gang members who rotate through each corner to attend Youth Night at "The Temple," a social program sponsored by Eastwood Community Church. In addition to owning and operating a community redevelopment company and a drug rehabilitation center, the church has numerous other social programs. Every day of the week is dedicated to a different cause. In the church's Saturday night hip hop worship service, young Eastwood residents perform gospel-rap hybrids; they sing and dance for the Lord; and they stage skits about certain circumstances that may test a teenager's faith. At the end of each service, Pastor Tim gives a sermon.

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On Mondays, in preparation for the Saturday service, all of the teenagers and young adults associated with the event meet for "Temple Practice." After a thirty-minute Bible study, those in attendance break off into small groups: Pastor Tim meets with the planning committee in the church library, the gospel rappers meet in the conference room, and the interpretive dancers along with young men who engage in a Chicago-style dance called "footworkin'" share the day-care center. While the "Temple Leaders" who help run these meetings are typically in their twenties and thirties and may have been involved with gangs at some point in their lives, the majority of the teenage students who attend Temple Practice would be considered upstanding youths—except for the young men who "footwork."

Footworkin' is a seemingly chaotic style of dance characterized by fast movements of the feet. It is specific to Chicago. In the 1980s, street gangs created it during the explosion of violence that coincided with the drug boom; quickly, it became one of the many expressions of gang rivalry. The frenzied, fleet-footed quickstep is performed by frenetically stomping or kicking one's feet in a stutter-stepped pattern. It fuses African dance moves with tap dance and is comparable to "krumping" in Compton, or "chickenhead" in St. Louis—distinctive regional outgrowths of break-dancing. Whereas break-dancing movements highlight flow, layering, and ruptures in line (movements in which the joints are snapped abruptly into angular positions to create a "semi-liquid effect"), footworkin' prizes rupture over fluidity. Footworkers vigorously dribble their sneakered feet in ways that capture the repetitive, hyper-accelerated samples of a local musical genre known as "juke."[11]

In recent years there have been efforts to capitalize on the appeal of footworkin'. Dancers have posted their latest moves on the Internet, and one group of street dancers, the FootworkKINGz have made two documentaries on the genre, which they circulate via the web. Despite these efforts, footworkin' remains a marginalized social practice—particularly because of its association with gang life.

The footworkers who attend Temple Practice are not all gang members. In many instances, though, the best footworkers are. Because the high school dropout rate is so high in Eastwood (71 percent), footworkin' gang members often have a surplus of free time. At any hour,

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day or night, a person might see them outside practicing their craft. Additionally, footworkin' is closely associated with underground "juke" parties, which are often the refuge of high school dropouts in Eastwood. (The upstanding student, who also happens to footwork, is far less likely to attend.) Footworkers from particular streets and belonging to certain gangs are thought to be consistently better than those from other areas; understandably, this sense of rivalry has been known to spark feuds.

It is not surprising, then, that less than a month after Pastor Tim invites street dancers to participate in the hip hop worship service, gang-affiliated footworkers begin attending Temple Practice. Not only does this dress rehearsal provide a venue for footwork battles between various male-dominated crews; the footworkers also get to use the church's state-of-the-art sound system. And during Chicago's harsh winters, the dancers are afforded shelter and heat. There is a catch, though: they have to sit through Bible study.

He never formally theorized the ritual of Bible study, but in examining the spheres of social life that produce the kind of order in which certain rituals are based, Emile Durkheim defined religion as such: A unified system of beliefs and rituals that unite a "moral community."[12] Indeed, part of the power of religious-based rituals is that everyone participates; even if someone is a nonbeliever, his or her presence does not detract from the spiritual potency of the service. In Pastor Tim's Bible study, however, the principle that Durkheim describes works in reverse.

The footworkers frequently disrupt biblical teachings with groans, sighs, and jokes. They spend the majority of their time signaling to one another in coded language, not so subtly communicating that they would rather be elsewhere. Even the upstanding footworker is pressured to, say, giggle mischievously alongside his more demonstrative peers.

Two months into my volunteer work at Eastwood Community Church, I am in the day-care center with Fatima Kearns, a teenager who does interpretive dance. She is showing me several interpretations of a dance sequence she plans to perform. Church groups from Portland, Oregon, to San Antonio, Texas, have staged versions of it, and as I watch alongside Fatima on the same desktop that the footworkers use to master the latest dance moves, I begin to notice how each interpretation

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centers on which temptations a given congregation should avoid. Just as Fatima begins outlining her plans for the performance, a rumble of laughter interrupts our YouTube session. On the other side of the door, the footworkers are making a spoof out of Fatima's cherished song. Tiko stands on his toes and waves his arms dramatically, pretending to be a ballerina. The rest of the footworkers cackle. One by one, they form a queue behind Tiko, flailing their limbs and eventually collapsing.

The following week, another, more serious incident occurs. It begins when the forty or so footworkers who have come for Temple Practice are asked by Pastor Tim to state their name and what school they attend. The footworkers start to snicker after Tiko says that he doesn't even go to school, and Pastor Tim, visibly annoyed, instructs Tiko to separate from the boys he came with.

"I wasn't doing anything," Tiko says. "So I shouldn't have to move."

"Either move or leave," Pastor Tim says.

Next, Tiko walks out, and a stream of teenage boys follows his lead. Soon only three footworkers remain. Shortly thereafter, we hear a familiar ruckus: a crowd of footworkers engaged in a dance battle outside of the church. Pastor Tim approaches the glass door and waves them off. "Go home," he yells.

Tiko, in the heat of battle against a young man named John Sage, jitters forward toward the door, pauses, allowing everyone watching to become transfixed by the rhythmic flashes of his gym shoes, and then kicks the door violently, shattering the glass. The footworkers flee. It takes a couple of weeks (and a fainthearted apology) before street dancers are permitted to return to Temple Practice. In the interim, church patrons debate the pros and cons of continuing to grant gang members use of their facilities. It is clear that for some churchgoers, footworkin' is a malevolent social practice that fuels gang rivalries and violence; for others, it's a way to avoid gang activity by channeling aggression into an expressive outlet.

The more time I spent at the Temple, the more I began to understand: it is because footworkers are habitual—yet far from ideal—churchgoers that their sporadic dances discomfort the institutional processes of moral transformation. Even if a footworker identifies as Christian, he or she is likely to be skeptical of the church's self-conscious, reflexive narrative

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of transformation. Many gang members I talk with read the advice of church congregants as criticisms of their actions, their affiliations, and the geography of commerce in which they circulate. (Oftentimes, this is true.) On the other side of the argument, many churchgoers feel that the gang's drug-dealing network—and the intravenous injection needles that come along with it—are the two major reasons diseases like HIV are prevalent in Eastwood. In the context of the church, footworkin' symbolizes their wider disdain with gang activity.

And to make matters worse: Every Monday the footworkers disrupt Bible study with uncannily consistent outbursts. (The Temple Leaders say: "If they hate being here so much, if they are *so unhappy*, they should just *go home*.") On many occasions, in fact, the footworkers are so unruly that they're dismissed before they can take advantage of a booming sound system and a warm place to dance—their two key incentives for being at Eastwood Community Church. And yet every week they come anyway, even though the vast majority of them do not attend the church service on Saturday that they are supposedly practicing for.

The crucial point: When urban residents who harbor disdain for the gang are the same people who inhabit positions of authority in neighborhood institutions or programs (such as the Temple), gang members often feel censored. The gang members' protests, from Tiko's well-aimed kick to many more subtle outbursts, correspond to an index of injuries they feel and the ways they feel that the church broadly paints them and their actions as reckless.

The door still isn't fixed by Saturday evening. Jagged shards of shattered glass are frozen in the frame, a disfigured snowflake. Just before the Youth Night festivities start, Pastor Tim replaces the fractured glass with a large piece of wood. In addition to the fifteen or so footworkers who attend Youth Night, the church has a large and loyal constituency of young people in their congregation (around sixty); tonight, in total, nearly one hundred teenagers are in attendance. Kids from nearby churches who occasionally attend the Temple, one of the only churches in the area to have a Saturday-night service, bolster the crowd.

"Tonight we're gonna talk about the temptations that test our commitment to God," Pastor Tim begins. "I want to start with a dance per-

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formance. Everyone, please welcome our performers with a round of applause."

As the pastor exits, the interpretive dancers I have watched rehearse for weeks take the stage. Fatima is, perhaps, the most dedicated of them all. She organized this week's interpretive dance rehearsals, and Pastor Tim is especially happy to have her engage in activities at the church. A little over a month ago, Fatima's older brother, Steve Kearns, a twenty-two-year-old member of the Divine Knights, hung himself in his prison cell after being denied parole. Because of the recent tragedy she's endured, Pastor Tim hopes that surrounding her with a supportive community will provide the emotional foundation Fatima needs to tackle an arduous grieving process.

Tonight's performance is about developing the resolve it takes to heal from injury. It begins with Fatima lying on her back onstage. Two other Temple Leaders are behind her: Peter Olson, wearing a brown vest and army fatigues; and Olivia Arnold, dressed in black pants and a black hooded sweatshirt. Only Blake Pietz, who's playing the role of the Christ figure, has yet to make an appearance; when he does, robed in white, he stands behind Fatima, placing his hand above her head. She rises. He then aims both hands to the ground, keeping them parallel while raising them vertically: a tree trunk. He picks an imaginary piece of fruit from this tree and then gives Fatima a bite. Blake takes her hand and they dance, their movements choreographed to a contemporary gospel song called "Everything," by the band Lifehouse: "Find me here, / And speak to me / I want to feel you, / I need to hear you / You are the light / That's leading me / To the place / Where I find peace again."

I half expect Tiko to be ridiculing the performance just as he did Fatima's rehearsal, but, to my surprise, the footworkin' gang member watches with what appears to be reverence.[13] Onstage Peter, the young man in the brown vest and army fatigues, cuts in on Fatima and Blake's dance. Littering the stage with dollar bills, Peter leads her away from the Christ figure. Fatima drops to recover the bills. A young woman behind me blurts out, "No, girl. Don't let him con-trol you." Fatima collects the money and pretends to be satisfied, smiling just as brightly as when the Christ figure gave her fruit.

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But Peter soon snatches the money, and Fatima, clearly dismayed, turns from him only to be greeted by Olivia, the young woman in the black hooded sweatshirt. Olivia is carrying a knife. She demonstrates how to take the blade to her wrist; Fatima follows Olivia's lead, slicing both before dropping the knife. She tries to push Olivia away, but Olivia pulls a gun from her waistband and points it at our protagonist. She doesn't shoot, though. Instead, Olivia hands Fatima the gun, who turns the barrel toward her own head and is about to pull the trigger. Just as the song reaches its crescendo, however, Fatima slams the gun down and jumps in the direction of the Christ figure. The Lifehouse song booms:

> 'Cause you're all I want
> You're all I need
> You're everything, everything
> You're all I want
> You're all I need
> You're everything, everything

Music blaring, the antagonists who visited Fatima onstage wrestle for control over her horizontally outstretched body. She is surfing on the extended arms of her enemies, all of whom are pushing her backward. As Fatima struggles with the villains, I have to admit that I am moved by her performance, despite its heavy-handed symbolism. Fatima's struggle with the Grim Reaper and her rejection of suicide are reminders of how hard it must be for her to deal with her brother's death. Judging from the lack of stray chatter, I can only assume that some of the teenagers—many of whom know Fatima personally and are aware of her loss—feel the same way.

Crying out to Jesus onstage, Fatima's efforts to reconnect with him are palpable. Her face is covered with sweat, her biceps bulging. As the villains repeatedly push her backward, they rip her shirt to shreds. Just as the tempters descend on her exhausted body, the Christ figure reenters, shielding her from the villains. He fends them off while Fatima throws both arms in the air, relieved. The audience applauds, and Blake helps her to a standing position. Fatima's friends do not even wait until

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she's offstage to congratulate her: they whistle and yell, unambiguously indicating that they found the performance riveting.

Pastor Tim meets her in the aisle and gives her a hug. "Now, that would've made Steve proud," he says. Overhearing him, I am reminded that Youth Night performances are designed to aid in the cultivation of a communal sense of healing. Indeed, the Lifehouse song "Everything" calls out to God repeatedly, imploring him to speak to the congregation, to lead them to peace. And there is no knowable timeline for when a person will attain that peace. She must submit to God's will in order to find it—this is the message.[14] Notice that unlike Tiko and the footworkers, Fatima is not vigorously contesting the church's narrative of transformation. Even so, the absence of a knee-jerk dismissal does not mean Fatima accepts the narrative wholeheartedly. By taking it upon herself to choreograph a dance for others to witness, she is making sense of what it means to transform—to heal—in the context of her own life.

Once the applause dies down, Pastor Tim taps the microphone, ready to make an announcement that, as I interpret it, means the following: As a part of the institutional nexus interconnecting an array of influential local organizations, each of which is interested in social reform, Eastwood Community Church is obliged to make a Christian understanding of healing and injury compatible with another nonprofit that deals specifically with health concerns. "Right now I have something extremely important to say," Pastor Tim says. "We have the good people from Healing Hearts with us. And we want to let all of you know that behind the stage, inside of the church library, you can be tested for HIV. You can be tested right here."

Chicago has one of the highest rates of HIV in the nation, Pastor Tim explains. Worse, Eastwood has one of the highest rates of HIV in Chicago. "Believe it or not, all of you are especially susceptible to this disease. You're young. You're black. And you grew up right here." He adds, "You all—well, all of us—we're in the middle of a fight right now, a fight to save our communities, our families, our lives. Knowing your status is a key weapon in this fight. Know your status. I beg you."

Days before Youth Night, I heard Pastor Tim inform the Temple

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Leaders to get tested for HIV along with the teenage participants. He wanted to send the message to the teenagers that there's no stigma attached to getting tested. After his public service announcement tonight, Pastor Tim encourages us to follow him to the church's gymnasium, which Healing Hearts volunteers have set up as an HIV-testing site. Everyone in the crowd leaves the sanctuary. Today, in solidarity with those who are brave enough to enter the testing center, I wait with about eighty young congregants. A single bedsheet serves as a door. It offers little barrier between an adolescent and the line of her peers waiting patiently for a health worker to wave them in, swab the inside of their mouth, and, twenty minutes later, inform them of their status.

On this night, two girls and two boys are told that they are HIV positive. They attended Youth Night presumably to have a good time and left knowing they are infected with a chronic virus. Unfortunately, these urban youths are proof positive that injury is pervasive in urban communities like Eastwood—and that it can emerge without warning.

"These are what I call blue blessings," Pastor Tim says. He feels anguish, he tells me, but is also grateful that tonight nearly eighty teenagers got tested. Meanwhile, Mark focuses on caring for the newly diagnosed. He is worried about the initial bout of depression they will likely endure. He stresses his commitment to stay in touch with these youth, and the hard work it will take to make sure they do not get swept up in denial or disconnect from their loved ones. Or worse, attempt to escape the reality of their diagnosis with alcohol and narcotics.

"I want to help them come to terms with the fact that they have HIV—you know, the way I did with Amy," Mark says. "'Cause accepting their positive status as a part of who they are is the first major challenge of their new lives." The new life of the patient to which Mark refers dovetails with the Christian concept of being born again. The difference is that rebirth will be grounded in injury. It entails the rigorous management of a chronic disease.

In Eastwood, healing—both in the religious and medicalized sense of the word—is processed and interpreted in different ways. In some cases, injuries are detected through biomedical technologies that analyze DNA from the inside of a person's cheek; in others, the collective

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experience of injury is based more on empathy (as in Fatima's performance, which could be viewed through the prism of her brother's suicide).[15] Here, modes of examining injury, as opposed to being at odds with one another, reveal that help is mutually constitutive. Community leaders like Mark and Pastor Tim depend on the injured to convey the missions of their respective institutions. (In addition, the injured also help keep these institutions attuned to social issues that are relevant to the neighborhood.) I learn this lesson—and others—about a month after Youth Night, when I accept Pastor Tim's invitation to attend the Rebirth Center Breakfast.

An important annual community event, the Rebirth Center Breakfast celebrates the men who have successfully matriculated in and transitioned out of the church's drug-rehabilitation program. At the breakfast I run into Noel Reyes, a former official in the Divine Knights Gang. When I first began my research, I interviewed Noel about his previous position in the gang. Noel was born and raised on James Street, and he was a childhood friend of Kemo, Red, and Shawn. Although he didn't have Kemo's prowess, Red's charisma, or Shawn's pedigree, he was an astute thinker. His job was preparing and assigning "G-packs"—large quantities of prepackaged heroin or cocaine that street-level dealers sell on the blocks that the Divine Knights control—to gang leaders. Because new members are continuously recruited into the fold to sell narcotics, the person responsible for providing them drugs to distribute and determining who gets what kind of product (marijuana or heroin, prescription drugs or cocaine) has substantial power. Ten years ago Noel lost a prestigious job, and nearly his life, when he began using the heroin that he was supposed to be divvying up for his fellow affiliates.

Six months after I arrived in Eastwood, Noel confided in me, while describing his fall from a high-ranking Divine Knight to a drug addict. "That job meant everything to me," he said. "I was on top of the world, making real money for the first time in my life. It meant everything." Noel shook his head, as if trying to erase the memories of the days when he privileged the gang over what he now holds dear: God and his family.

Now he sits among the proud alums of the Rebirth Center, preparing to deliver his speech, his notes beside a freshly polished plate of chicken and waffles. One of the highlights of the annual banquet is that spouses

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of these men don smocks and hairnets, serving up all the food their husbands can eat. Pastor Tim begins the event with a brief introduction, after which Monica performs a medley of gospel music. Adorned in an all-white choir robe, her voice fills the air, aligning the injury of addiction with a notion of spiritual redemption: "Redeemed, how I love to proclaim it / Redeemed by the blood of the Lamb / Redeemed through His infinite mercy / His child, and forever I am."

When Monica finishes her performance, Noel approaches the podium. A recent graduate from the Rebirth Rehabilitation Center where he was a resident for twelve months, Noel addresses current participants about what they can do with their lives after they graduate. "The most important thing," he says, "is to have a plan. You gotta know what you want to do. But you can't just want any old job. You gotta be familiar with the qualifications it takes to get a particular kind of job. You may need college. But, then again, you may not. The first step is to find out exactly what kind of training you need."

At first, Noel paces in an upright posture. Soon, though, he grabs a stool from the corner of the stage, sits down, and relaxes—a signal, perhaps, that he is preparing the audience for a dialogue (a conversation among a group of peers, many of whom don't know what it is like to reenter society after leaving a rehab clinic). It's a dialogue that's much less formal than a Pastor Tim sermon.

"Most of you know that I want to be a chef," Noel continues. "I'll be finishing my associate's degree in culinary arts in a couple of months." This is met with resounding applause. "My favorite dish to prepare is Cajun-baked catfish. It's the dish my wife likes most. But I had to master this dish, you understand? I had to cook it over and over. At first I put in too much paprika. Then too little garlic. See the balance has to be just right to stimulate the taste buds. And that only happens by preparing a dish over and over again."

Noel is building toward an analogy: The same discipline required to learn to cook well is also required to wake up every day and surrender to God's will. As Noel makes this connection, Pastor Tim smiles. He seems pleased that Noel is emphasizing God's role in preparing him, psychologically, for rehabilitation. "I was lost when I first came to the

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Rebirth Center," Noel says. "I thought I knew it all. And let me tell you, it's hard for someone like me who was a gang leader back in the day to humble himself and let someone tell me when to eat, when to go to bed, and how to spend my money."

Continuing, Noel says, "You see, in the Rebirth Center, after you get rid of that monkey on your back, you're eligible to get a job in the community. I started working at Big Al's, the soul food restaurant down the street. I remember when I first got paid, Pastor Tim took my check. It was the first legitimate money I had made in a long, long time. And he said I couldn't spend it. He said that he would give me what I needed for the week and he would save the rest."

A self-described headstrong "grown-ass *man*," Noel didn't easily submit to such treatment. ("How you gonna tell me how I can spend *my* own hard-*earned* money.") Pastor Tim allotting Noel his money was, somewhat ironically, a reversal of the power that Noel once had over young gang members. Prior to Rebirth, the last time someone rationed Noel's money stream he was likely a fresh-faced Divine Knight hoping to make a name for himself.

"I was really upset," Noel says. "But then I remembered the lessons we learned in the Rebirth Center about letting go—the lessons about letting God take over. And it was a blessing, because a couple months after I had that argument, I got hit with child support. The only reason—and I mean the *only* reason—that I was okay was because I had all of that money saved up."

Noel, acknowledging that he should've been paying child support all along, scans the room, trying to gauge everyone's facial expressions. "But you gotta understand," he says, "I had a resentful character before. I was extremely childish. The way I acted with Pastor Tim was the same way I acted with everybody. There were times when my child's mother asked for money—times when she tried to get places to garnish my wages. and I wouldn't let her do it. I've quit jobs before because she was taking money out of my check but wouldn't let me see my son."

Because he quit so many jobs, Noel was often broke. To maintain even a modicum of hygiene, he stole things like soap and bubble gum. Once he was caught for the petty theft. "I was in County and they

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were asking me, 'Hey, B, what you in for this time?' I said, 'Man, I had about five packs.'" I laugh with the crowd at the pun between bundles of heroin (G-Packs) and bubble gum.

Wrapping up, Noel gets serious once again. "All jokes aside, I owe everything to the Rebirth Center and to my family for sticking by my side. To my wife, Grace, I thank you for helping to change me into the man I am today. We men don't know what these women go through, putting up with our mess. Grace probably didn't know what she was getting into."

"No way," Grace Reyes yells from behind the serving table. "But I'd do it all over again."

Noel's testimony illustrates the importance of a "renegade will" as a resource for resilience. Such performances of redemption—performances in which urban residents display a willingness to heal—are the basis for institutional collaboration. What's more, his speech shows that institutional efforts to help people heal from disease and addiction culminate at the very moment an urban resident internalizes a story of redemption, makes it his or her own, and a polished, vigorously rehearsed illness narrative becomes an asset for others. At Eastwood Community Church, part of making a resource out of personal pain and suffering means teaching others to develop the necessary willpower to recover from injurious events. Noel now considers his tenure as a gang member misguided, his previous exploits destructive. These were the years when his rank in the Divine Knights determined his self-worth, when he was too "childish" to see the value in providing for his family.

Once again Eastwood Community Church proves to be a site where medical notions of healing (in terms of managing addiction) and religious notions of healing (in terms of maintaining faith in God) are fostered, and the reiteration of a willingness to heal becomes a critical component in this process. Only this time, instead of a crowd full of teenage boys and girls, the audience is male drug addicts—many of whom are former gang members—and their families, who can't hear too many times that the people who once disappointed them are willing to change. Here, the discipline and patience it takes to replicate a meal

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again and again is consonant with the discipline and patience deemed necessary to harness injury in order to heal.

Three weeks after the Rebirth Center Breakfast, Pastor Tim decides to take this message of redemption and communal healing beyond the church walls. On Monday night he unveils his idea to the Temple's planning committee: a worship service in the style of a block party on a stage on Murphy Road, right in front of the church. There will be food and large speakers to entice residents to leave their homes, and, because she can bridge the gap between believers and nonbelievers, Amy will headline the pastor's event.

By the time Amy is due to speak on an unseasonably warm Saturday in March, around two hundred residents are seated in foldout chairs in front of the makeshift stage. For the past hour, Eastwood Community Church's choir has been performing. Another fifty or so Eastwoodians stand in the general vicinity of the church, making idle conversation. Amy has spoken at the church before (she has even told her story at a local middle school and a recreation center), but this is by far her largest audience. Standing behind Pastor Tim, she appears nervous. She is positioned to tell the crowd about the resolve it takes to manage injury on a daily basis. "Hello, everyone," Amy says, taking the mic with far less confidence than the seasoned preacher. "I'm eighteen years old and I'm here to talk about how I contracted HIV, the virus that causes AIDS." As she emerges from the shadows, I realize that I haven't seen her since she was released from the juvenile detention center.

Growing up, Amy was a tomboy, she says. She loved playing basketball, which she learned from her mom. "She was my best friend. The turning point in my life came when I was fourteen and my mother died." Here, Amy's throat cracks, but she continues: "My mother, Sue-Ann Green, was six months pregnant when she was killed in a drive-by shooting. The baby also died. Her boyfriend was a member of the Knights. He was also a drug dealer. And one day someone decided to take his life. My mother happened to be with him, and he used her as a human shield. He lived. She didn't."

Recounting the circumstances of her mother's death in a staccato of

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measured sentences, Amy's eyes remain fixed on her spiral notebook. Later Amy tells me that when she goes off script she becomes teary-eyed and uncontrollably impassioned, unable to make it through her speech.

After her mother's death, Amy lived with her aunt. But as soon as she moved there, her aunt started using drugs. Heroin was her drug of choice. In her defense, Amy says of her aunt, "She used to say that she was so distraught over my mom's death that she turned to drugs to deal with her grief and pain. I don't make those kinds of excuses. But somewhere down the line, I started using too. I would steal drugs and needles from my aunt; and I was only sixteen when I used heroin for the first time."

One day Amy woke up, walked into the bathroom, and found her aunt on the ground, dead of a drug overdose. After she died, Amy began going to a drug-rehabilitation center. Before starting treatment, she was tested for sexually transmitted diseases, the results of which revealed that she was HIV positive. "I still don't know how I contracted the virus," she says. "It could've been from having sex. It could've been from sharing needles with my aunt. I think it was from my aunt because she was skinny; and toward the end of her life she started getting sores and abrasions on her skin—"

As Amy is speaking, I hear a familiar voice punctuating her words. Behind me, Mark, her health worker and friend, has once again come to lend Amy moral support, delivering words of encouragement like, "Yes, that's right," and, "Please, child, tell 'em." As he deploys the call-and-response interactions that accompany Sunday sermons to affirm the diagnostic techniques that he's taught Amy, medical and religious discourses intersect once again.

"Nobody knows whether or not my aunt had AIDS," Amy adds. "But the point is: I don't want to die young like her. Or like my mother." Rolling up her left sleeve to expose her forearm, she says, "You probably can't see this. But it's a tattoo. This is a tattoo of my mother, and it covers up the places where I used to inject. As an addict, I can't think about death. Thinking about death is easy. Death doesn't even scare me. It's living that's hard. For me, every single day is a struggle."

Amy's story personalizes an already-valuable healing narrative. As someone who is well equipped to discuss an onerous and ongoing pro-

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cess of healing, she is an expert at eliciting empathy. On the one hand, Amy's illness narrative is similar, formally, to Fatima's dance performance: both are built on a framework that would be familiar to an audience of churchgoers. But, like Noel's speech, the point of it all is not merely personal transformation, but to focus the audience's attention on the daily rigors of managing injury. Like "renegade dreams," the "renegade wills" that Fatima, Noel, and Amy have developed are grounded in injury and seek to refigure the pain experienced through addiction and disease into newfound aspirations or a radical reorientation to the world (in this case related to sobriety and drug adherence).

By making a story of redemption her own, Amy reminds us all that, as an HIV patient, survival means a lifelong commitment to adhere. The resiliency that Amy has developed makes multiple institutional understandings of healing commensurate. As such, both believers and nonbelievers can make sense of the willpower cultivated in this communal space. In this way, Amy's narrative speaks to how people in Eastwood make sense of volition in light of the kind of injuries you have to think about every day for the rest of your life (or at least until your will to adhere becomes stronger than the disease that threatens it). In this context, developing the will it takes to cope with the injury of addiction and disease ceases to be solely a single-minded cognitive relationship with a higher power. It is a far more radical endeavor. Allowing other Eastwoodians to see you managing various forms of injury is both action and strategy that can be adopted in adverse times—a collective asset others might draw on. This is the very resource on which many of their lives may depend.

The day after Amy's speech, I visit her at a group home for girls where she now lives. While I am there, discussing the impact of Amy's illness narrative, Mark arrives. Sitting on the shabby steps of a group home with Mark and Amy, I witness the narration of experience, as Mark helps Amy develop the language she needs to attune herself to any side effects of her antiretroviral treatment. He encourages her to talk with her physician about the side effects of her cocktail therapy (nausea, sluggishness). He reminds her to schedule doctor's appointments, since doing so will allow her doctor to change the drug regimen in relation to the progression of the disease. His efforts are crucial. The will that

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Mark encourages Amy to harness and maintain is both a general power to overcome obstacles in life, and also specifically related to adhering to a drug-treatment regimen.

Moments later we watch Mark drive away. Then Amy reminds me of our first encounter in cellblock 3B, just over a year ago, though it seems like an eternity, she says, because of what she's learned in the meantime. As we watch the overcast sky turn bright, she confesses that a sense of willingness is rebooted in her every day at noon—when there's no nurse in flowery scrubs to check under her tongue, and she shuts off her cell phone reminder, opens her purse, and swallows seven pills that, she hopes, will sustain her.

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